

TAMS MUN I

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World Health Assembly

The Black Market in Human Organs

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Letter from the Chair

Dear delegates,

Welcome to the inaugural session of TAMS Model United Nations, held on 26 September at the Business Leadership Building at the University of North Texas. On behalf of your dais, it is our privilege to chair this committee.

Every year, somewhere between 100,000 and 150,000 organ transplants are performed worldwide. That covers well under 10% of the global need. The gap between those two numbers is the entire topic. Where a legal system cannot supply an organ to a person who will otherwise die within months, an illegal one appears, and it is supplied by people who are poor.

This committee is not primarily a law-enforcement body — that work belongs to UNODC and to national criminal justice systems. WHO's leverage is different and, in some ways, larger: it sets the ethical standards that professional bodies enforce, it runs the data systems that make the trade visible, and it is the only forum where health ministries argue about the thing that actually drives the market, which is **organ scarcity**.

A good resolution here does two things at once: it makes trafficking harder, and it makes trafficking unnecessary. A resolution that only does the first will push the trade into a darker corner, and delegates should expect the dais to ask about that.

A note on tone. This topic involves coercion, poverty, and the exploitation of refugees and prisoners. Debate it as a public health and human rights problem. Graphic detail, jokes, and lurid speculation are not in order and the dais will rule them so.

Sincerely,

Your Chair

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1. What the Market Actually Looks Like

The dominant commodity is the living-donor kidney. A person can survive with one, the operation is comparatively routine, and demand is enormous because dialysis is expensive, miserable, and finite in effectiveness. Livers (partial lobes from living donors) are the second market. Hearts and lungs require deceased donors and cannot be trafficked in the same way — which is why claims about “organ harvesting” from live victims are far rarer than the market in *purchased* kidneys from consenting-but-coerced-by-poverty sellers.

The commonly cited estimate is that roughly 5–10% of transplants worldwide involve organs obtained illicitly. That range traces to work published by WHO researchers in 2007 and has been repeated ever since; delegates should cite it as an estimate of unknown precision, because by definition nobody counts an illegal transplant. WHO’s **Global Observatory on Donation and Transplantation (GODT)** collects what legal activity states report; the illicit share is inferred.

The typical structure of a transaction:

- A **broker** recruits a seller in a poor community, often through debt, often through a returning former seller.
- A **recipient**, usually from a wealthier country, pays a package price — historically tens of thousands of dollars, of which the seller receives a small fraction, and frequently less than promised or nothing.
- A **medical team and facility** perform the operation. This is the essential point: *organ trafficking requires doctors*. It cannot happen without surgeons, nephrologists, anesthesiologists, tissue-typing labs, and a hospital. Every documented case has involved licensed professionals.
- Documentation is falsified to present the seller as an altruistic relative or emotionally related donor.

Documented hotspots and cases across recent decades include commercial kidney networks in South Asia, transplant tourism corridors into several countries in the Middle East and South-east Asia, the **Medicus clinic case in Kosovo** (re-

cipients flown in from multiple countries, sellers recruited from Eastern Europe and Central Asia; convictions obtained by an EU rule-of-law mission), the exploitation of Syrian refugees in Lebanon reported by multiple journalistic and NGO investigations, migrant vulnerability along North African routes, and longstanding, contested allegations regarding the sourcing of organs from executed prisoners in China, which announced in 2015 that it had ceased the practice and now operates a national allocation system (COTRS).

The pattern across all of them: **sellers are poor, recipients are mobile, and the middle of the chain is a licensed hospital.**

Outcomes for sellers are bad. Follow-up studies of kidney sellers in South Asia consistently find that most did not escape the debt that motivated the sale, that self-reported health declined, that income fell, and that a large majority regretted it. They rarely receive aftercare, because seeking it means admitting to a crime.

2. The Legal and Normative Framework

- **WHO Guiding Principles on Human Cell, Tissue and Organ Transplantation** — first adopted 1991, updated and endorsed by **WHA63.22 (2010)**. Eleven principles. The core ones: consent; priority to deceased donation; prohibition on organs being the subject of financial gain (Principle 5, which permits reimbursement of legitimate expenses and lost earnings but not payment); transparent and traceable allocation; and physician responsibility not to transplant an organ they have reason to believe was obtained commercially.
- **Declaration of Istanbul on Organ Trafficking and Transplant Tourism (2008, revised 2018)**. Not a treaty — a professional consensus document, now endorsed by well over a hundred medical societies. It supplies the working definitions of *organ trafficking*, *transplant commercialism*, and *travel for transplantation* that the field uses. Its custodial group (DICG) is a major actor.
- **UN Protocol to Prevent, Suppress and Punish Trafficking in Persons (Palermo Protocol, 2000)**. Includes “the removal of organs” as a

purpose of trafficking in persons — which criminalizes trafficking the *person*, but not necessarily trafficking the *organ* where no person was moved. UNODC leads here.

- **Council of Europe Convention against Trafficking in Human Organs (Santiago de Compostela, 2015; in force 2018).** The most important recent instrument: it criminalizes the illicit *removal* of organs directly, plus solicitation, recruitment, brokering, transport, and use — and it is **open to non-member states of the Council of Europe**. Ratifications remain modest. Pushing universal accession is one of the most concrete asks available to this committee.
- **WHA57.18 (2004), WHA76 discussions**, and successive WHO reports on transplantation; the **Global Observatory (GODT)**, run with Spain's ONT.

3. The Upstream Problem — Why Supply Is Short

Trafficking is a symptom of scarcity, and scarcity is a policy choice as much as a biological fact. Deceased-organ donation rates vary by more than a factor of fifty between countries with similar medical capability. Spain sustains roughly 45–50 deceased donors per million population; many high-income countries manage a third of that; most low- and middle-income countries report near zero.

What Spain actually did — the model most often invoked and most often misunderstood — was not simply pass an opt-out law. It built a national transplant organization (ONT) with a **transplant coordinator physician in every hospital ICU**, paid for detection and referral of potential donors, systematic family approach training, and continuous audit of every death in every ICU for donation potential. Presumed consent is on the books but is rarely invoked against family objection. **The lesson is that infrastructure, not the consent default, produces donors** — a point delegates proposing opt-out legislation should be ready to defend.

Other levers on the supply side: paired kidney exchange chains, expanded criteria and donation-after-circulatory-death protocols, normothermic

machine perfusion to make marginal organs usable, national registries and unique donor identifiers, dialysis and prevention investment to reduce incident end-stage renal disease at all, and — over a longer horizon — xenotransplantation and bio-engineered organs, which are advancing but are not a 2020s policy answer.

4. The Central Controversy — Prohibition vs. Regulated Compensation

Delegates should expect this to dominate debate, and both sides have serious arguments.

The prohibitionist position (WHO Guiding Principles, Declaration of Istanbul, most member states): payment for organs is inherently exploitative because only the poor will ever sell; it converts the body into a commodity; it crowds out altruistic and deceased donation; and no regulatory system in a country with meaningful inequality can make the consent real.

The regulated-market position points to **Iran**, which has operated the world's only legal compensated living kidney donation system since 1988 — state-regulated, with a fixed government payment plus charitable supplement, brokered by a patient association rather than by private brokers. Proponents note it eliminated the kidney waiting list. Critics note that donors are overwhelmingly poor, that follow-up and long-term outcomes are weak, that foreign recipients are excluded so the model is untested as an international system, and that reported donor regret and health outcomes resemble those of illegal sellers elsewhere.

Intermediate ground that resolutions often occupy: **removing financial disincentives** — reimbursing lost wages, travel, childcare, and providing lifelong health insurance and follow-up for living donors — which the Guiding Principles already permit and which most states have not implemented.

5. Case Files

Case File 1 — The Medicus Clinic (Kosovo, 2008)

The best-documented prosecution in the field. Sellers — recruited from Moldova, Ukraine, Russia,

Turkey, and Central Asia with promises of up to \$20,000 — were flown to Pristina, where kidneys were removed at the Medicus clinic and transplanted into recipients from Israel, Canada, Germany, and Poland who paid up to \$130,000 each. Most sellers were paid a fraction of what was promised or nothing. The scheme unraveled when a Turkish seller collapsed at Pristina airport. An EU rule-of-law mission (EULEX) prosecution convicted five, including the clinic's owner and chief urologist. What the case proves: an international trafficking chain requires visa facilitation, an accredited clinic, licensed surgeons, and tissue-typing — every link is regulable, and every link failed.

Case File 2 — Iran's regulated model, in mechanism

Since 1988: a candidate seller and a matched recipient are introduced through the Dialysis and Transplant Patients Association (DATPA), a patient NGO — not a private broker. The state pays a fixed award and one year of health insurance; the recipient's family customarily adds a negotiated supplement (historically ~\$2,000–\$4,000 equivalent). Foreigners are barred from the system entirely, and vendors and recipients must share nationality. Result: the kidney waiting list was eliminated by 1999 — the only country to achieve this. The critiques delegates must engage rather than wave at: studies of Iranian vendors find over 70% are poor, majorities report negative health and employment outcomes, follow-up is weak, and the fixed award has lagged inflation, pushing the real price into the informal supplement. The Iranian delegate's strongest line: every one of those criticisms applies *worse* to the illegal markets operating everywhere prohibition is the policy.

Case File 3 — Spain's model, in mechanism

The Organización Nacional de Trasplantes model, built from 1989: a physician *transplant coordinator* in every ICU (usually an intensivist working part-time in the role), paid detection of every potential donor, mandatory audit of every hospital death for missed donation opportunities, professionalized family-approach conversations, and

24/7 national allocation logistics. Deceased donation rose from 14 to ~49 donors per million population — while "opt-out" consent, on the books since 1979, is in practice never enforced against family refusal. Croatia imported the model wholesale in the 2000s and went from bottom of Europe to near the top in a decade; Portugal and parts of Latin America followed. Cost estimates for the coordinator network are modest relative to dialysis: a year of hemodialysis costs a health system several times a transplant's marginal cost, so donation infrastructure is one of the rare interventions that *saves* money. The question for this committee is why, three decades on, most member states still have not copied it — and what WHO can do about the answer, which is: it requires ICU capacity many systems do not have.

Case File 4 — Numbers every delegate should know

- Kidneys account for roughly two-thirds of all transplants worldwide; the WHO-cited estimate that 5–10% of transplants involve illicit organs dates to 2007 and has never been superseded by better data — cite it as an estimate.
- A seller in South Asia has typically received \$1,000–\$3,000; the recipient package price runs \$50,000–\$200,000. The spread is the broker's and the clinic's.
- Follow-up studies in Chennai and Pakistan: ~85–95% of sellers report health decline, most remained in or returned to debt, and roughly three-quarters would not recommend selling.
- Global dialysis population: ~4 million and growing ~5–6% annually; the transplant gap widens every year even where trafficking is zero.
- The Council of Europe Convention (CETS 216) has been ratified by fewer than 20 states. It is open to every UN member. Universal accession is the cheapest concrete ask available to this committee.

6. Questions to Consider

1. How should states be encouraged to criminalize the *purchase and brokering* of organs, in-

- cluding when committed by their nationals abroad (extraterritorial jurisdiction)?
2. Should organ sellers be treated as victims rather than offenders, and what protection and aftercare follows from that?
 3. What role should professional accountability play — licensing sanctions, mandatory reporting by transplant units, refusal of follow-up care as a pressure point, journal and society sanctions?
 4. What should a mandatory national **transplant registry and vigilance system** contain, and should states be required to report travel-for-transplantation to WHO?
 5. How can WHO support deceased-donation infrastructure in low- and middle-income countries — coordinators, ICU capacity, family consent training — and who funds it?
 6. Should the WHA call for universal accession to the Santiago de Compostela Convention?
 7. What is this Assembly's position on regulated compensation, and can a resolution acknowledge the disagreement without collapsing into meaninglessness?

8. How are follow-up care and insurance guaranteed for living donors, including those who donated illegally?

7. Sources and Further Reading

- WHO Guiding Principles (WHA63.22) — read all eleven; they are two pages and will improve every position paper
- Declaration of Istanbul (2018 edition) and the DICG website
- GODT / IRODaT data for your country's donation and transplant rates — a required number for your position paper
- Council of Europe Convention (CETS 216) and its ratification list
- UNODC, *Trafficking in Persons for the Purpose of Organ Removal* (2015)
- Scheper-Hughes, "The Global Traffic in Human Organs"; Shimazono, *Bulletin of the WHO* (2007), for the origin of the 5–10% figure
- Global Financial Integrity, *Transnational Crime and the Developing World* — for market-size estimates, used with caution